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Health service experiences of gender- and sexually diverse intended parents throughout the surrogacy journey: a scoping review protocol

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ABSTRACT

Objective: This scoping review will identify and map health-related literature concerning gender- and sexually diverse individuals engaging with surrogacy in health care settings, including the stages of the surrogacy journey, and any reported barriers and facilitators.

Introduction: Gestational surrogacy involves engagement with multiple health services and professionals across fertility, maternity, birth, and postnatal care, where cis-heteronormative assumptions about people's identities, family structure, and parenthood persist. Evidence indicates that gender- and sexually diverse individuals often report poorer experiences in health services, compared with cisgender heterosexual individuals. Bias, discrimination, and the absence of inclusive information are commonly reported issues. There is limited guidance on how health services should support this group, which may contribute to variability in practice and suboptimal outcomes.

Eligibility criteria: This scoping review will include scholarly and gray health-related literature concerning gender- and sexually diverse individuals engaging in surrogacy within health services. Literature focused solely on legal or ethical issues without health application will be excluded. There will be no restrictions on location, language, or publication date.

Methods: The review will be guided by the JBI methodology for scoping reviews and reported using the Preferred Reporting Items for Systematic Reviews and Meta-Analyses extension for Scoping Reviews (PRISMA-ScR). Published literature will be located using CINAHL (EBSCOhost), Emcare (Ovid), Health Collection (Informit), and MEDLINE (Ovid). References of included studies will be hand-searched. Gray literature will include government and professional college documentation obtained from websites. Findings will use a combination of graphical, diagrammatic, and/or tabular formats accompanied by narrative explanations.

Review registration: OSF https://osf.io/72n46/

Keywords: delivery of health care; gestational carriers; health services; reproductive techniques, assisted; sexual and gender minorities

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Introduction

Families are changing faster than the health care systems and policies that serve them. Over the past decade, advances in assisted reproductive technologies and shifting social attitudes have made gestational surrogacy an increasingly viable pathway to parenthood for people who cannot carry a pregnancy themselves. In Australia and New Zealand, surrogacy use has grown steadily, with births resulting from surrogacy more than tripling over the past decade.[1,2] Similar increases in surrogacy use over time have also been observed in the United States.[3] Previous research has shown that surrogacy is increasingly accessed by a diverse range of intended parents, including some

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gender and/or sexually diverse (GSD) individuals and couples.[4-6] The term GSD is an inclusive term encompassing identities commonly referred to as LGBTIQA+ (lesbian, gay, bisexual, transgender, intersex, queer/questioning, asexual/aromantic, and other gender, sex, or sexualities).

Despite the growth in surrogacy births, health systems, policies, and everyday clinical practices often remain shaped by cis-heteronormative assumptions. Research across Australia, the United States, and Canada shows that GSD individuals face additional hurdles in health systems, including inconsistent recognition as the parent, fragmented or inappropriate care pathways and resources, forced identity disclosure and/or misgendering, and bias and discrimination during pregnancy, birth, or early parenting years.[7-9] These issues can alienate GSD individuals from health services, contribute to psychological distress, and are associated with poorer experiences and outcomes internationally[7,10,11] and within Australia.[9,12] Such challenges are often intensified within surrogacy, where care occurs across multiple settings and involves complex relationships between intended parents, surrogates, and health care professionals. While surrogacy is described as a collaborative, team-based process,[13] the complexity of surrogacy and lack of health guidance for professionals[14] can expose gaps in how health systems recognize and support diverse family forms.

Surrogacy is a form of assisted reproduction in which a person, referred to as a *gestational carrier* or *surrogate*, carries and gives birth to a child for another individual or individuals, known as the intended parents, to raise the child.[15] Surrogacy can be classified as traditional or gestational. In traditional surrogacy, the surrogate's own oocyte is used, meaning the surrogate has a genetic relationship with the child. *Gestational surrogacy* is performed via in vitro fertilization (IVF), where gametes from one or both intended parents, or from donor(s), or any mix of these, are fertilized in vitro to form a zygote that develops to an embryo.[16] The embryo is then transferred to the surrogate. As gametes were not taken from the surrogate, there is no genetic relationship between the surrogate and the child. Surrogacy *arrangements* may be altruistic, commercial, or hybrid/compensated.[17] Altruistic arrangements have no commercial reward or profit for the surrogate but may include reimbursement to cover basic costs.[17] In Australia, only altruistic surrogacy is legally permitted.

A growing body of primary research has examined surrogacy involving GSD intended parents, particularly gay male couples, across fertility treatment, pregnancy, birth, and early parenthood. Some of these studies have explored inadequate preconception counseling and informed consent processes,[18] the importance of GSD intended parents being present during prenatal care (eg, ultrasound, with surrogate consent),[19] how parenthood is shaped through prematurity or critical illness,[19] negative experiences in fertility clinics or hospitals,[8,18,19] need for psychological and/or tailored support services (eg, counseling, GSD support groups),[8,18,19] and the need for training of health care professionals.[8,19] Many findings reflect the emotional and practical work required to navigate health systems shaped by implicit bias and cisgender heterosexual family norms. Other primary studies have examined health care professionals' perspectives and the broader clinical, ethical, and relational complexities of surrogacy care.[4,6] However, relatively few studies are explicitly focused on GSD intended parents, and where they are included. Findings are often embedded within broader populations. As a result, the evidence base is not well oriented to care delivery for GSD intended parents within health services, with recommendations often remaining broad (eg, improve staff education), offering limited practical guidance.

Collectively, the primary research suggests that a sufficient but fragmented body of evidence exists, supporting the need for a scoping review to map the literature, clarify its characteristics, and identify gaps relevant to health care practice improvement. By mapping and describing the available evidence, the review aims to identify what is currently known and where gaps in knowledge remain regarding the support of GSD intended parents across the surrogacy journey.

A preliminary search of MEDLINE, the Cochrane Database of Systematic Reviews, *JBI Evidence Synthesis*, the JBI Systematic Review Register, PROSPERO, OSF, and Google Scholar did not identify any in-progress or previously conducted systematic or scoping reviews focused on GSD intended parents engaging with surrogacy in health settings. Previous reviews have focused on the psychosocial experiences of both surrogates and (any) intended parents,[20] experiences in the delivery of preconception and pregnancy care for GSD individuals,[21] and concerns with commercial surrogacy.[22] In contrast, the proposed review

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will concentrate on surrogacy use by GSD intended parents in health and health care settings, and will describe how care is delivered, communicated, and structured across surrogacy-related health encounters, offering more targeted insights into how affirming care can be delivered across the surrogacy journey for GSD intended parents.

A scoping review methodology was selected to provide a descriptive map of the available evidence and identify key characteristics and relevant sources.[23] Establishing the current evidence base is an important first step in identifying gaps in knowledge and highlighting areas that may warrant further research relating to affirming care for GSD intended parents during surrogacy. Where surrogacy health resources exist, they may not be explicitly labeled or designed for GSD people. Non-specific or irrelevant information, or assumed heterosexuality, can contribute to variability in care and poorer health experiences and outcomes for GSD individuals.[7,8,11] Poorer experiences or outcomes may reflect that health services lack policies or care pathways that recognize diverse family structures, with health professionals relying on fragmented or generic resources. Calls to improve surrogacy health care more broadly have already been made[14]; however, it remains important to consider the specific needs of minority groups within this pathway to parenthood.

Review questions

What evidence is available on how GSD intended parents engage with, experience, and navigate health services, including access to health information and resources, during the surrogacy journey?

Sub-questions:

i) How is the identified literature distributed in terms of research focus, type of publication, geographic location, and methodological approaches?
ii) Which stages of the surrogacy journey are addressed by the identified evidence, from preconception to early postnatal care?
iii) What health information, resources, and guidance are identified in the literature with relevance to GSD individuals engaging with surrogacy?
iv) What barriers and facilitators are reported in relation to providing inclusive health services to GSD individuals during surrogacy?

Eligibility criteria

Participants

The primary population of interest is GSD adults (≥18 years) who are intended parents involved in surrogacy. For the purpose of this review, GSD refers to individuals whose gender identity, sex characteristics, or sexual orientation differ from cisgender and heterosexual norms. Studies will be included where GSD individuals are described in relation to their engagement with, experience of, or navigation of health services during the surrogacy journey. Studies involving health care professionals will be included if they describe the provision of care or services to GSD individuals.

Included studies may focus entirely on one group of GSD people or many groups. Studies including mixed populations (eg, GSD and cisgender heterosexual participants) will be included only where data relating to GSD participants are reported separately. Where data cannot be disaggregated, the study will be excluded. The review will exclude studies focused on children or adolescents (<18 years), or studies focused on only cisgender and heterosexual individuals, with no inclusion/consideration of GSD individuals.

Concept

The review will focus on surrogacy that involves GSD individuals. *Surrogacy* is defined as a form of assisted reproduction in which a surrogate carries and gives birth to a child for another individual or individuals (intended parents) to raise. Studies will be included regardless of surrogacy type (traditional or gestational) or surrogacy arrangement (altruistic, commercial, or compensated/hybrid).

In this review, the *surrogacy journey* is conceptualized as a series of stages spanning preconception to early postnatal care. These stages may include; but are not limited to, pre-surrogacy counseling, fertility treatment and embryo creation, intended parent and/or surrogate screening processes, pregnancy and antenatal care, birth, and the early postnatal period (up to 6 weeks post-birth). Six weeks postpartum was selected as it represents the conventional postnatal care period during which routine maternal and newborn follow-up commonly occurs after a birth.

The review will consider evidence relating to how GSD individuals engage with, experience, and navigate

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health services across the stages of the surrogacy journey. This includes, but is not limited to, interactions with health care providers and services; models of care and service delivery; access to, and provision of, health information, education (dedicated or group); and tailored GSD resources relevant to surrogacy.

In addition, the review will capture reported *barriers and facilitators* of GSD individuals engaging with and accessing health services during the surrogacy journey. Barriers may include structural (eg, insurance/funding), organizational (eg, policy), interpersonal (eg, attitudes, bias, or discrimination), or informational (eg, consumer resources) barriers that limit access to, or quality of, service. Facilitators may include positive practices by staff (eg, affirmative and inclusive), resources (eg, tailored documentation, information, or groups), or other factors that highlight inclusive, accessible, and responsive service provided to GSD individuals. Studies will be excluded if they do not focus on surrogacy.

Context

The context of interest is health services, including both public and private services. Health services include, but are not limited to, community services (eg, home care, general practice, mental health, maternal health), specialist services (eg, fertility clinics, sexual health) and hospital health care services (eg, birthing suite, inpatient care, outpatient services). Sources will be included where interaction with health services is explicitly described, or where health involvement is clearly embedded within a participants' broader surrogacy journey (eg, retrospective account of experience across the surrogacy journey).

Where a health context is not clearly specified in the abstract, full-text review will be undertaken to determine eligibility. Sources that do not involve interaction with, or implications for, health services (eg, legal or ethical-focused sources) will be excluded. This review will consider studies conducted in any geographical location.

Types of sources

This scoping review will consider both reviews and primary studies. Any type of design is acceptable, including quantitative, qualitative, or mixed methods. If primary studies are identified that are already included within an eligible evidence synthesis, both studies would be included, but data would only be extracted once. The scoping review will also consider other literature, such as health policies, procedures, protocols, guidelines, and education/information materials.

Methods

The proposed scoping review will be conducted in accordance with the JBI methodology for scoping reviews[24] and reported using the Preferred Reporting Items for Systematic Reviews and Meta-Analyses extension for scoping reviews (PRISMA-ScR).[25] A Peer Review of Electronic Search Strategies (PRESS) was performed by an academic librarian, as per the Table 10: PRESS guideline assessment form.[26] This scoping review has been registered with the OSF (https://osf.io/72n46/).

Search strategy

Database searches for literature will be conducted, followed by a gray literature search. An initial limited search of MEDLINE (Ovid) and Google Scholar was undertaken to identify articles on the topic. The text words contained in the titles and abstracts of relevant articles, and the index terms used to describe the articles were used to develop a search strategy. Following this, the search strategy was refined in consultation with an academic librarian, with Medical Subject Headings (MeSH) explored to map key concepts to appropriate headings, and the PubMed PubReMiner v1.31 (Jan Koster, Amsterdam, The Netherlands) used to identify and refine alternative search terms. A full search strategy for MEDLINE (Ovid) was undertaken in May 2026 (see Appendix I). The search strategy, including all identified keywords and index terms, will be adapted for each included database: CINAHL ultimate (EBSCOhost), Emcare (Ovid), Health Collection (Informit), and MEDLINE (Ovid). The reference list of all included sources of evidence will be screened for additional studies. Studies published in any language will be included, and no date limits will be applied. Where potentially eligible sources are published in languages other than English, DeepL (DeepL, Cologne, Germany) will be used to translate titles, abstracts, and, where necessary, full texts to support screening and data extraction. There will be no further verification of the translations.

Many professional guidance documents are never published in peer-reviewed journals, yet they directly influence health care delivery and consumer experiences. A comprehensive search of gray literature from

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every jurisdiction globally is not feasible. Therefore, a purposive sample of English language countries with established health services (where either altruistic and/or commercial surrogacy is legally permitted) were selected. Gray literature will be obtained from all national-level government health, surrogacy, or fertility websites; and up to 3 professional colleges/societies from each of the countries selected, acknowledging that some countries share colleges/ societies (Appendix II). The keyword "surrog*" will be used where supported on sites' internal search function, otherwise the terms "surrogacy," "surrogate," and "gestational carrier" will be searched and items screened against the eligibility criteria. The absence or presence of relevant material from the gray literature that relates to the eligibility criteria will be reported in the scoping review.

Sources of evidence selection

Following the search, all identified citations will be collated and uploaded into EndNote v21.5 (Clarivate Analytics, PA, USA), using a separate group to store the results from each database. Citations will be imported into Covidence (Veritas Health Innovation Ltd, Melbourne, Australia) to commence screening and remove duplicates. A pilot test of at least 25 titles and abstracts will be conducted by the review team, and screening will commence once at least 75% agreement is achieved.[24] Titles and abstracts will then be screened by 2 or more independent reviewers for assessment against the eligibility criteria for the review. Potentially relevant sources will be retrieved in full and their citation details imported into Covidence. The full text of selected citations will be assessed in detail against the eligibility criteria by 2 or more independent reviewers. Reasons for exclusion of sources of evidence at full text that do not meet the eligibility criteria will be recorded and reported in the scoping review. Any disagreements that arise between the reviewers at each stage of the selection process will be resolved through discussion or with an additional reviewer. The results of the search and the selection process will be reported in the final scoping review and presented in a PRISMA flow diagram.[27]

Data extraction

Data extraction will start with a pilot test. Reviewer one will extract the data from 3 sources, then 2 other reviewers will provide independent feedback and verification. Any discrepancies in data extraction or limitations identified in the data extraction form will be discussed by the review team and resolved by consensus. If these result in changes to the data extraction form, the pilot test will be repeated. Data will be extracted from sources included in the scoping review by reviewer one using a tool developed by the review team (see Appendix III). A second reviewer will verify the data extracted, with any discrepancies resolved through discussion or consultation with a third reviewer. Extracted data will include details about the evidence source, including participant characteristics (eg, GSD), surrogacy journey, identified barriers or facilitators, and key findings. The draft data extraction tool will be revised as needed during data extraction, with any modifications reported in the review. Authors will be contacted once to request missing or additional data, if required.

Data analysis and presentation

Data will be presented using tabular, graphical, and diagrammatic formats, supported by a narrative summary. Findings will be mapped across the surrogacy journey within health services to illustrate areas of concentrated and limited research, including through matrix tables or heat-map-style visualizations. Extracted data will be descriptively grouped by key characteristics, such as participant focus (eg, broadly inclusive vs targeted GSD populations) and study focus (eg, experiences, education, guidelines), with categories applied flexibly and refined during analysis. The accompanying narrative will contextualize the findings, describe patterns and gaps, and link results to the review objective and research question, outlining what evidence is available, for whom, and at which stages of the surrogacy journey.

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Government will not be involved in the conduct or reporting of the review.

Global priority and equity alignment

This review aligns with Sustainable Development Goal 3.7 (sexual and reproductive health). By mapping evidence on GSD individuals' experiences of health services during surrogacy, we aim to promote person-centered universal access for GSD people to sexual and reproductive health services.

Author contributions

CJF was responsible for the concept, drafting, review, editing, and finalization of the protocol. MP assisted in the development of early drafts and all subsequent reviews. All authors contributed to discussions, reviewed drafts, provided feedback, piloted the data extraction tool, and approved the final draft.

References

1. Macaldowie A, Lee E, Chambers GM. Assisted reproductive technology in Australia and New Zealand 2013 [internet]. The University of New South Wales; 2015 [cited 2026 Jan 9]. Available from: https://researchers.mq.edu.au/en/publications/assisted-reproductive-technology-in-australia-and-new-zealand-201-2/.
2. Kotevski DP, Newman JE, Chaitarvornkit A, Paul RC, Chambers GM. Assisted reproductive technology in Australia and New Zealand 2023 [internet]. The University of New South Wales; 2025 [cited 2026 Jan 9]. Available from: https://www.unsw.edu.au/content/dam/pdfs/medicine-health/npesu/research-reports/2025-09-anzard/2025-09-Assisted-Reproductive-Technology-in-Australia-and-New-Zealand-2023.pdf.
3. Perkins KM, Boulet SL, Jamieson DJ, Kissin DM. Trends and outcomes of gestational surrogacy in the United States. Fertil Steril 2016;106(2):435-42.e2.
4. Kneebone E, Hammarberg K, Everingham S, Beilby K. Australian intended parents' decision-making and characteristics and outcomes of surrogacy arrangements completed in Australia and overseas. Hum Fertil (Camb) 2023;26(6):1448-58.
5. Everingham SG, Stafford–Bell MA, Hammarberg K. Australians' use of surrogacy. Med J Aust 2014;201(5):270-3.
6. Kneebone E, Hammarberg K, Beilby K. Surrogates', intended parents', and professionals' perspectives on ways to improve access to surrogacy in Australia. Int J Law Policy Fam 2024;38(1).
7. Coulter-Thompson EI, Matthews DD, Applegate J, Broder-Fingert S, Dubé K. Health care bias and discrimination experienced by lesbian, gay, bisexual, transgender, and queer parents of children with developmental disabilities: a qualitative inquiry in the United States. J Pediatr Health Care 2023;37(1):5-16.
8. Fantus S. Experiences of gestational surrogacy for gay men in Canada. Cult Health Sex 2021;23(10):1361-74.
9. Haire BG, Brook E, Stoddart R, Simpson P. Trans and gender diverse people's experiences of healthcare access in Australia: a qualitative study in people with complex needs. PLoS One 2021;16(1):e0245889.
10. Alday-Mondaca C, Lay-Lisboa S. The impact of internalized stigma on LGBT parenting and the importance of health care structures: a qualitative study. Int J Environ Res Public Health 2021;18(10):5373.
11. Lisy K, Peters MD, Schofield P, Jefford M. Experiences and unmet needs of lesbian, gay, and bisexual people with cancer care: a systematic review and meta-synthesis. Psychooncology 2018;27(6):1480-9.
12. Brener L, Broady T, Cama E, Treloar C. The impact of sources of stigma on health care avoidance among gay and bisexual men in Australia. AIDS Care 2023;35(5):663-71.
13. Jefford S. More than just a baby: a guide to surrogacy for intended parents and surrogates. Indie Experts; 2020.
14. Attawet J, Qiu Y, Peters M. Towards best practice: urgent need for surrogacy birth care guidelines in Australia. Aust J Adv Nurs 2024;41(4):1-3.
15. GovernmentAustralian. Terminology: A glossary explaining key surrogacy terms and concepts [internet]. Australian Government; n.d. [cited 2026 Jul 14]. Available from: https://www.surrogacy.gov.au/terminology.
16. Betts JG, Young KA, Wise JA, Johnson E, Poe B, Kruse DH, *et al.* Fertilization. Anatomy and physiology 2e. OpenStax; 2022.
17. Attawet J, Brady M, Sattarshetty K, Jefford S. The ethical and legal implications of commercial surrogacy for healthcare: a global perspective, 1st ed. Routledge; 2025.
18. Riggs DW, Due C, Power J. Gay men's experiences of surrogacy clinics in India. BMJ Sex Reprod Health 2015;41(1):48.
19. Biton N. The transition to parenthood through surrogacy among Israeli gay fathers: barriers, resources, and formative time points. J Homosex 2025;73(12):1-23.
20. Kneebone E, Beilby K, Hammarberg K. Experiences of surrogates and intended parents of surrogacy arrangements: a systematic review. Reprod Biomed Online 2022;45(4):815-30.
21. Permezel J, Arnold AS, Thomas J, Maepioh AL, Brown R, Hafford-Letchfield T, *et al.* Experiences in the delivery of preconception and pregnancy care for LGBTIQA+ people: a systematic review and thematic synthesis of patient and healthcare provider perspectives. Midwifery 2023;123:103712.
22. Attawet J, Alsharaydeh E, Brady M. Commercial surrogacy: landscapes of empowerment or oppression explored through integrative review. Health Care Women Int 2025;46(10):1081-99.
23. Peters MD, Godfrey C, McInerney P, Khalil H, Larsen P, Marnie C, *et al.* Best practice guidance and reporting

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items for the development of scoping review protocols. JBI Evid Synth 2022;20(4):953-68.
24. Peters M, Godfrey C, McInerney P, Munn Z, Tricco A, Khalil H. Scoping reviews. In: Aromataris E, Lockwood C, Porritt K, Pilla B, Jordan Z, editors. JBI Manual for Evidence Synthesis [internet]. JBI; 2024 [cited 2026 Jan 29]. Available from: https://synthesismanual.jbi.global.
25. Tricco AC, Lillie E, Zarin W, O'Brien KK, Peters MD, Straus SE. PRISMA extension for scoping reviews (PRISMA-ScR): checklist and explanation. Ann Intern Med 2018;169(7):467-73.
26. Drug AgencyCanada's. PRESS guideline — search submission and peer review assessment. [internet]. CDA; 2016 [cited 2026 Jan 23]. Available from: https://www.cda-amc.ca/press-peer-review-electronic-search-strategies.
27. Page MJ, McKenzie JE, Bossuyt PM, Boutron I, Hoffmann TC, Mulrow CD, *et al.* The PRISMA 2020 statement: an updated guideline for reporting systematic reviews. BMJ 2021;372:n71.

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Appendix I: Search strategy

Ovid MEDLINE(R) ALL [1946 to May 12, 2026]		
Search conducted: May 14, 2026		
No.	**Searches**	**Records retrieved**
1.	exp "Sexual and Gender Minorities"/	24,074
2.	Bisexuality/	5259
3.	exp Homosexuality/	37,963
4.	Transsexualism/	4839
5.	exp "Gender Identity"/	29,350
6.	Health Services for Transgender Persons/	225
7.	(agender* or asexual* or bisexual* or bi sexual or gay or gays or glbt* or gender identity or gender identities or gender fluid or genderfluid or gender non confirming or gender nonconfirming or gender dysphoria or homosexual* or homo sexual or intersex* or inter sex or lesbian* or lesbigay or lgb* or nongender* or non gendered or nonbinary or non binary or non heterosexual or nonheterosexual* or queer* or same sex or samesex or transsexual* or transexual* or trans sexual or transgender* or trans gender or trans man or trans woman or trans men or trans women or transwom* or trans feminine or transfem* or trans masculine or transmasc* or transpe* or trans person or trans people or two spirit or twospirit*).ti,ab,kf.	116,915
8.	((gender or sexual*) adj3 (divers* or minorit*)).ti,ab,kf.	17,265
9.	or/1-8	159,086
10.	Gestational Carriers/	1952
11.	exp "Reproductive Techniques, Assisted"/	88,367
12.	(((gestation* adj2 (mother* or carrier*)) or host mother* or surrogat* or surrogac* or assisted reprod* or infertility therap*).ti,ab,kf.	110,445
13.	or/10-12	183,082
14.	9 and 13	1446

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Appendix II: Sample of government websites and professional organizations

	Government websites	Professional organizations (up to 3 per country)
Australia	Department of Health, Disability and Ageing (https://www.health.gov.au/) Surrogacy in Australia (https://www.surrogacy.gov.au/)	The Royal Australian and New Zealand College of Obstetricians and Gynaecologists* (https://ranzcog.edu.au/) Fertility Society of Australia and New Zealand* (https://www.fertilitysociety.com.au/) Australian College of Midwives (https://www.midwives.org.au/)
Canada	Health Canada (https://www.canada.ca/en/health-canada.html)	Society of Obstetricians and Gynaecologists of Canada (https://sogc.org/) Canadian Fertility and Andrology Society (https://cfas.ca/) Canadian Association of Midwives (https://canadianmidwives.org/)
New Zealand	Health New Zealand/ Te Whatu Ora (https://www.healthnz.govt.nz/) Ministry of Health/ Manatū Hauora (https://www.health.govt.nz/) Advisory Committee on Assisted Reproductive Technology (https://acart.health.govt.nz/)	New Zealand College of Midwives (https://www.midwife.org.nz/) *Shared organizations listed under Australia*
United States	Department of Health and Human Services (https://www.hhs.gov/) Centers for Disease Control and Prevention (https://www.cdc.gov/) National Institutes of Health (https://www.nih.gov/)	American College of Obstetricians and Gynecologists (https://www.acog.org/) American Society for Reproductive Medicine (https://www.asrm.org/) Society for Assisted Reproductive Technology (https://www.sart.org/)
United Kingdom	National Health Service (https://www.nhs.uk/) Department of Health and Social Care (https://www.gov.uk/government/organisations/department-of-health-and-social-care) Human Fertilisation and Embryology Authority (https://www.hfea.gov.uk/)	Royal College of Obstetricians and Gynaecologists (https://www.rcog.org.uk/) College of Sexual and Reproductive Healthcare (https://www.cosrh.org/) Royal College of Midwives (https://rcm.org.uk/)

*The college/society is shared.

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Appendix III: Draft data extraction instrument

Scoping review details			
Review title			
Review objective			
Review questions			
Eligibility criteria			
Population			
Concept			
Context			
Types of evidence source			
Evidence source details and characteristics			
Citation details (eg, author/s, date, title, journal, volume, issue, pages)			
Participants (demographics, inc. gender and sexual diversity)			
Health context (specify)			
Country			
Details/results extracted from source of evidence			
Key details	At what stage of the surrogacy journey does the evidence relate to?	What is the evidence about?	What are the barriers and facilitators described (if any)?
	☐ Counseling ☐ Pre-conception/ child planning ☐ Early pregnancy (first trimester) ☐ Mid-pregnancy (second trimester) ☐ Late pregnancy (third trimester) ☐ Birth ☐ Postnatal/postpartum ☐ Other	☐ Experience(s) ☐ Consumer health information ☐ Health professional information ☐ Review ☐ Other	

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